

Travel health Action Sheet

Peru · Lagos, Nigeria → Lima, Peru · 90 days · Leisure
May 11, 2026

SECTION 1: CRITICAL ACTIONS BEFORE DEPARTURE

- Book a pre travel health consultation with a travel medicine clinic or your primary care physician as soon as possible, ideally at least 6 to 8 weeks before 21 May 2026
- Begin the rabies pre exposure prophylaxis series immediately (3 doses: days 0, 7, and 21); this is the most time sensitive vaccine requirement
- Receive yellow fever vaccination at an approved centre at least 10 days before departure; obtain your International Certificate of Vaccination (yellow card)
- Receive hepatitis A vaccination at least 2 weeks before departure; begin hepatitis B accelerated series (days 0, 7, 21 with 12 month booster)

SECTION 2: TRIP SNAPSHOT

Alone, 90 days, Leisure, overall risk HIGH

SECTION 3: VACCINES

No qualifying vaccines listed in dossier.

SECTION 4: WHAT TO PACK AND DAILY ROUTINE

- Malaria prophylaxis: Atovaquone proguanil (Malarone) or doxycycline, to be determined by clinician based on specific itinerary within Peru. Malaria transmission occurs in Peru, primarily in jungle and rural regions (Loreto, Ucayali, Madre de Dios, and other Amazonian departments). Plasmodium vivax is the predominant species; P. falciparum also occurs. Lima city itself is not a malaria risk area, but the traveller is staying in a rural environment for 90 days, which significantly elevates risk. The specific malaria risk depends on which rural areas are visited. Atovaquone proguanil is preferred for its tolerability and suitability for shorter defined periods, though for a 90 day stay cost may be a consideration. Doxycycline is a cost effective alternative for longer stays. Mefloquine is an option but should be avoided if there is any history of neuropsychiatric conditions. The clinician should review the specific rural itinerary to determine the most appropriate agent and whether prophylaxis is needed throughout the stay or only during higher risk phases.
- Pre travel review and vaccination records: Book a formal pre travel health consultation with a travel medicine clinic or your primary care physician as soon as possible, ideally at least 6 to 8 weeks before your 21 May 2026 departure. This is essential to allow time for multi dose vaccine series (hepatitis A, hepatitis B, rabies pre exposure) and for malaria prophylaxis to be prescribed and dispensed. Bring any existing vaccination records to the appointment. Confirm your routine immunisation history is complete and documented.
- Food and water hygiene: In rural Peru, treat all drinking water as potentially unsafe. Drink only bottled water with intact seals, or water that has been boiled or treated with iodine or chlorine tablets. Avoid ice in drinks unless you are certain it was made from safe water. Eat only thoroughly cooked food served hot. Avoid raw salads, unpeeled fruit, street food from unhygienic vendors, and unpasteurised dairy products. The rule 'boil it, cook it, peel it, or forget it' applies throughout your rural stay. Carry oral rehydration salts (ORS) for use if diarrhoea occurs. A standby antibiotic (such as azithromycin or ciprofloxacin) should be discussed with your clinician for self treatment of moderate to severe traveller's diarrhoea.
- Vector bite prevention: Peru has active transmission of malaria, dengue fever, Zika virus, and leishmaniasis, all transmitted by insect vectors. Use DEET repellent (30 to 50 percent)

on all exposed skin at all times outdoors. Wear permethrin treated long clothing. Sleep under a treated bed net in rural accommodation. Aedes mosquitoes (dengue and Zika) bite during the day; Anopheles mosquitoes (malaria) are most active at dawn and dusk. Maintain protection throughout the day. Sandflies (leishmaniasis) are active at night and in forested areas; fine mesh nets and repellent are essential. If you develop fever, rash, joint pain, or any unexplained illness during or after your trip, seek medical attention promptly and inform the clinician of your travel history.

- Sun, heat, and environmental precautions: UV radiation in Peru can be intense, particularly at altitude and in open rural terrain. Apply SPF 50 or higher broad spectrum sunscreen to all exposed skin and reapply every 2 hours and after swimming. Wear a wide brimmed hat and UV protective sunglasses. Stay well hydrated, particularly during hiking and outdoor activities. Avoid strenuous outdoor activity during peak heat hours (11am to 3pm). If you plan to travel to high altitude areas of Peru (above 2,500 metres, such as Cusco or the Andes), discuss altitude sickness prevention with your clinician before departure. Symptoms of acute mountain sickness include headache, nausea, dizziness, and fatigue. Gradual ascent and adequate hydration are the primary preventive measures; acetazolamide (Diamox) may be prescribed as a preventive medication.
- Injury and road safety: Peru has a high burden of road traffic injuries. Avoid travelling by road at night where possible, particularly in rural areas. Use reputable transport providers. Always wear a seatbelt. Avoid motorcycles and motorbike taxis (mototaxis), which are a common cause of serious injury among travellers. When hiking, use established trails, carry adequate water and supplies, inform someone of your route and expected return time, and do not hike alone in remote areas. Carry a basic first aid kit. Ensure your travel insurance covers emergency medical evacuation from rural areas, as medical facilities outside Lima may be limited.
- Sexual health and blood exposure: You have indicated you anticipate new sexual partners during this trip. HIV, hepatitis B, gonorrhoea, chlamydia, syphilis, and other sexually transmitted infections (STIs) are present in Peru. Use condoms consistently and correctly with every new partner. Carry an adequate supply of condoms from home, as quality and availability may vary in rural areas. Hepatitis B vaccination before departure is strongly recommended given this risk. If you believe you have been exposed to HIV (for example through unprotected sex), post exposure prophylaxis (PEP) must be started within 72 hours. Identify the nearest clinic or hospital in your area that can provide PEP before you need it, as access in rural Peru may be limited. Consider discussing PrEP (pre exposure prophylaxis for HIV) with your clinician before departure if you anticipate ongoing risk. Avoid tattooing, piercing, or any cosmetic procedures abroad unless you can verify sterile professional facilities. Avoid sharing needles or any blood contact equipment.
- Jet lag, sleep, and mental wellbeing: Lagos (WAT, UTC+1) and Lima (PET, UTC 5) are 6 hours apart. Expect some disruption to your sleep and circadian rhythm on arrival. Adjust your sleep schedule gradually in the days before departure. Stay well hydrated during the flight and avoid excessive alcohol. Expose yourself to natural daylight on arrival to help reset your body clock. A 90 day solo trip to a rural environment in a new country can be mentally demanding. Plan regular check ins with family or friends at home. Be aware of signs of culture shock, loneliness, or low mood, and have a plan for accessing mental health support if needed. Carry any personal medications you rely on in sufficient supply.

SECTION 5: RED FLAGS AND EMERGENCY CONTACTS

URGENT: Any fever occurring within 3 months of return from Peru must be assessed as a potential malaria emergency. Go to an emergency department or urgent care facility immediately, state that you have travelled to a malaria risk area, and request a malaria blood test. Do not wait for symptoms to resolve on their own.

CLINICAL FACILITIES

Clinica Internacional Lima, Jr. Washington 1471, Lima 15001, Peru

Clinica Anglo Americana, Alfredo Salazar 350, San Isidro, Lima 15073, Peru

LOCAL EMERGENCY NUMBER

105 (Police), 116 (Ambulance / SAMU)

INSURANCE

Insurance: Yes

Full clinical dossier attached. Share with your travel medicine clinician at your pre-travel appointment.